PART 6

Special Populations, Behavioral Emergencies and Safety

IN THIS PART

1. Describe the structured pediatric assessment and how children differ from adults.
2. Describe how choking relief, breathing support and CPR differ in infants and children.
3. Recognize pregnancy-related emergencies, describe safe positioning, and state when to call for help for an imminent birth.
4. Describe how assessment changes in older adults.
5. Describe a safe response to behavioral and mental health emergencies using safe-messaging language.
6. Recognize signs of abuse or neglect and state reporting duties in general terms.

Everything so far has used one patient as the model: an adult of average build, healthy until the emergency. Real patients vary. A child is not a small adult, a pregnant patient has two lives to protect, and an older adult may show a serious illness in an unex-

pected way. A person in distress of mind needs a different kind of care than a person in distress of body, and any patient may be a victim of harm that nobody has named.

This Part adapts the habits of Parts 1 to 5 to these patients. Entry 6.1 gives a way to assess a child and shows how children differ. Entry 6.2 covers the changes to choking relief, breathing support and CPR for infants and children. Entry 6.3 deals with pregnancy. Entry 6.4 covers older adults, Entry 6.5 covers behavioral and mental health emergencies, and Entry 6.6 covers abuse, neglect and violence.

Each entry adds only what differs. Where the adult method still applies, the text points back to the home entry and does not repeat it. What you may do still depends on your scope of practice and local protocol.

6.1 Children: Assessment and How They Differ

Children get sick differently from adults, and they compensate until late and can then deteriorate quickly. A child who is breathing hard and still playful is in a very different place from a child who is breathing hard and has gone quiet. This entry gives you a structured way to look at a child before you touch them, and then lists the main ways they differ.

WHAT IT IS. The pediatric assessment is a rapid, hands-off look at three things: the child's appearance, the work of breathing, and the circulation to the skin. It takes a few seconds, and it works from the doorway. Because it needs no equipment, it can be done with a frightened child still in a parent's arms. After it, you carry out the primary survey of Entry 1.3 in the same order as for an adult.

Figure 23 shows the three parts of the assessment; notice that the look is made from a distance, before you touch the child.

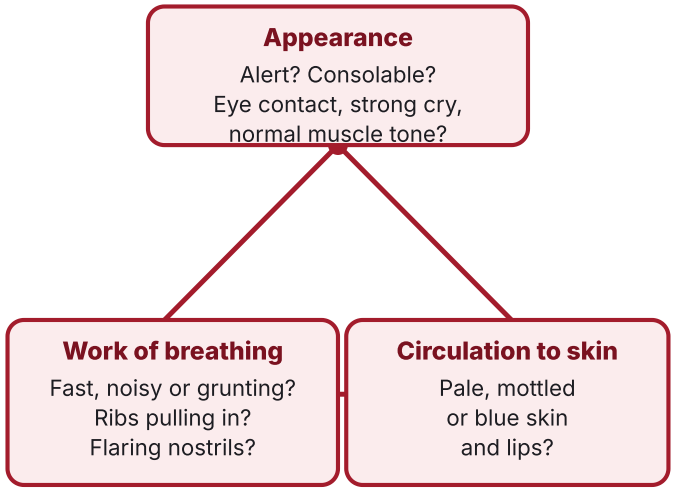


Figure 23. The three parts of the pediatric assessment: appearance, work of breathing and circulation to the skin.

APPEARANCE. Appearance reflects, among other things, whether the brain is getting enough oxygen and blood. Notice whether the child is alert, consolable and interested, whether they make eye contact, whether the cry is strong, and whether they move normally. A child who is limp, glassy-eyed, hard to wake or does not react to a parent is seriously unwell.

WORK OF BREATHING. Watch the child breathe. Look for fast or slow breathing, noisy breathing, flaring of the nostrils, grunting, pulling in of the skin between the ribs, and a child who sits in a tripod position, leaning forward on their hands.

CIRCULATION TO THE SKIN. Look at the color of the skin and lips. Pale, mottled or blue skin suggests poor circulation or poor oxygen supply. Compare it with the child's normal color.

TALKING WITH CHILDREN AND PARENTS. Get down to the child's level, use a calm voice, and let a small child stay in a parent's arms for as long as it is safe, since a frightened child is harder to assess. Ask the parent what is usual for the child, how long the illness has lasted, how much the child has drunk, how many wet diapers there have been, and what medicines and allergies the child has. A parent who says the child is not acting like themselves should be taken seriously.

DEHYDRATION. Children can lose fluid quickly through vomiting, diarrhea and fever. Signs include a dry mouth, few or no tears when crying, few wet diapers, sunken eyes, drowsiness and fast breathing. A child who is drowsy, cool and mottled with fast breathing is in shock and needs urgent help.

HOW CHILDREN DIFFER. Children differ from adults in several ways that matter in an emergency. Their airways are smaller and narrower, so a little swelling or mucus blocks a larger share of the passage. Their tongues are larger in proportion to their mouths. Their heads are large for their bodies, so a child who lies flat may have the neck bent forward, and a small pad under the shoulders may be needed to keep the airway in line. Entry 2.2 described the positions for adults. Children also breathe faster and have a faster heart rate than adults, and both change with age. The table below gives typical resting ranges by age.

Figure 24 compares the airway of a child with that of an adult; notice the size of the head and tongue relative to the airway.

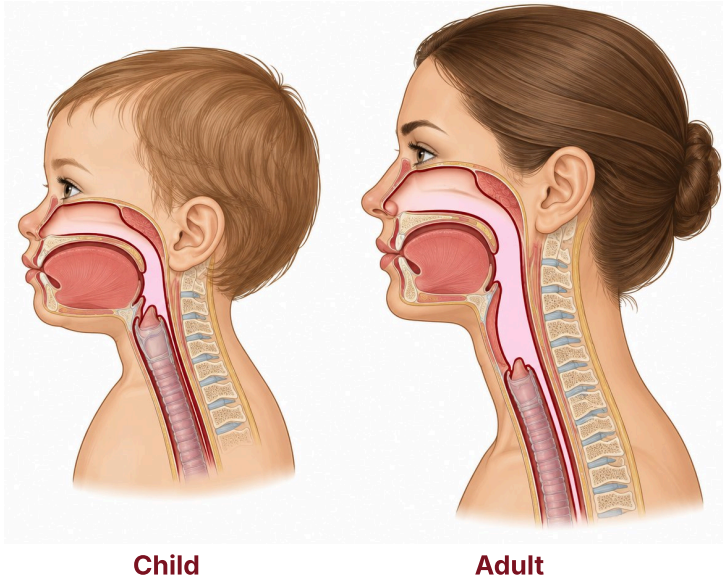


Figure 24. A child's airway compared with an adult's, drawn side by side.

Typical resting ranges in children

Age	Breathing rate (breaths per minute)	Heart rate awake (beats per minute)
Infant (1 month to 1 year)	30 to 53	100 to 190
Toddler (1 to 2 years)	22 to 37	98 to 140
Preschool (3 to 5 years)	20 to 28	80 to 120
School age (6 to 11 years)	18 to 25	75 to 118
Adolescent (12 to 15 years)	12 to 20	60 to 100

Sources differ slightly, and newborns in the first month have their own ranges; follow the values in your protocol.

Children compensate for illness and injury by speeding the heart and tightening the vessels, and may keep a normal blood pressure until they are close to collapse. A fast heart rate and a slow capillary refill are earlier signs than a low pressure. A child who becomes sleepy, breathes slowly or with less effort while still looking unwell, or whose heart rate falls is in danger; these are late and serious signs. Fever is common in children and by itself is usually not a danger sign, except in a baby under 3 months, where a temperature of 100.4 degrees F (38 degrees C) or higher needs urgent evaluation. A fever with a stiff neck, a rash that does not fade under pressure or purple bruise-like spots, extreme sleepiness, trouble breathing or poor feeding needs urgent care.

Several earlier entries gain one child-specific point. Asthma (Entry 2.4) shows the same signs in a child, who may tire sooner. A child has a small blood volume, so bleeding that would be minor in an adult can be serious, and an adult-sized tourniquet (Entry 4.2) may not fit a small limb; call for help early. Small children who are ill are at higher risk of low blood glucose, so a glucose check belongs in the assessment of any drowsy child, if your scope includes it (Entry 5.1). A first seizure with fever in a young child needs assessment (Entry 5.3), and any injury that does not fit the story needs attention (Entry 6.6).

URGENCY AND ALLERGY. Entry 1.2 set out the five levels of acuity. For a child the same levels apply, with one addition: a child who is limp, hard to wake or inconsolable is sorted higher whatever the numbers show, and a parent who says the child is not acting like themselves counts as a finding. Anaphylaxis (Entry 2.5) in a child may begin with hives, vomiting or a cry that turns hoarse rather than with obvious breathing trouble, so treat these after a likely trigger as an emergency. In sepsis (Entry 5.4) an infant may show only poor feeding, floppiness or an unusual cry, with or without fever. Burns (Entry 4.5) in a child need a clinician's assessment even when they look small, and the spinal precautions of Entry 4.3 apply to children with the padding adjustment described above.

RED FLAGS

A child who is limp, hard to wake or inconsolable, grunting or struggling to breathe, blue or gray, with a rash that does not fade, a stiff neck, repeated vomiting, or a fever in an infant under about 3 months. Call for help at once.

SCOPE NOTE

Children's equipment and medicine amounts are sized by weight and age and are outside the scope of this book. Use the sizing aids and protocols of your service, and call for help early for any seriously unwell child.

NURSING AND RESPONDER PRIORITIES. Look first from a distance, use the three-part assessment, involve the parent, and call early.

See also: Entry 1.3 (primary survey); Entry 2.4 (asthma); Entry 6.2 (child CPR); Entry 6.6 (abuse).

6.2 Children: Choking, Breathing Support and Resuscitation Differences

The principles of Parts 2 and 3 still apply, but the techniques change with the size of the child. This entry covers the differences, using the age groups that rescuers use: an infant is under 1 year, and a child is from 1 year to the start of puberty.

CHOKING IN AN INFANT. A choking infant has a weak or silent cry, a weak or silent cough, or cannot breathe. If the infant is coughing hard or crying strongly, do not give blows or thrusts; stay close and watch. Do not use abdominal thrusts on an infant.

PROCEDURE: RELIEVE CHOKING IN AN INFANT

Purpose: to clear a blocked airway in an infant under 1 year who cannot cry, cough or breathe. Scope: follows your training level and local protocol; the exact method your course teaches takes precedence.

1. Confirm that the infant cannot cry, cough or breathe, and shout for someone to call emergency services.
2. Lay the infant face down along your forearm, resting on your thigh, with the head lower than the chest.
3. Support the head and jaw with your hand, and keep your fingers off the soft part of the throat.
4. Give up to 5 firm back blows between the shoulder blades with the heel of your other hand.
5. If the object has not come out, turn the infant face up along your other forearm or on your thigh, with the head supported, and give up to 5 chest thrusts with the heel of one hand on the middle of the breastbone, just below the nipple line.
6. Look in the mouth after each round, and remove an object only if you can see it.
7. Repeat steps 2 to 6 until the object comes out or the infant becomes unresponsive.
8. If the infant becomes unresponsive, lay the infant on a firm surface and begin infant CPR (procedure below).

> **⚠ SAFETY**
>
> Do not use abdominal thrusts on an infant. Do not sweep the mouth blindly, because you may push the object deeper. Support the head throughout and keep it lower than the chest.

STOP AND CALL FOR HELP IF the object does not come out after several rounds, the infant turns blue or gray or goes limp, or the infant becomes unresponsive.

Back blows jolt the object loose, and chest thrusts raise the pressure inside the chest so that air pushes the object toward the mouth. Abdominal thrusts are avoided in infants because their organs sit low and are poorly protected by the ribs. Alternating the two gives each technique a chance to work.

CHOKING IN A CHILD. For a child who is conscious and cannot cough, speak or breathe, the sequence follows the adult method of Entry 2.2: alternate up to 5 back blows and up to 5 abdominal thrusts, with a force suited to the child's size. If the child becomes unresponsive, lower them to the ground, call for help, and begin CPR.

PROCEDURE: GIVE RESCUE BREATHS TO AN INFANT OR CHILD

Purpose: to give breaths to an infant or child who has a pulse but is not breathing normally. Scope: follows your training level and local protocol.

1. Make the scene safe, check for a response, and call for help; send someone for an AED.
2. Open the airway with the head in line with the body or tilted back only slightly (Entry 2.2).
3. Check for breathing, and for a pulse if your role includes it, for no more than 10 seconds.
4. Seal a mask of the right size over the nose and mouth (Entry 2.3 described the seal).
5. Give 1 breath every 2 to 3 seconds (20 to 30 breaths a minute), each lasting about 1 second, only enough to make the chest rise.
6. Recheck breathing and pulse about every 2 minutes, and start compressions if there is no pulse or you are not sure.

⚠ SAFETY

Give gentle breaths. Small lungs are easily over-filled, and air forced into the stomach can cause vomiting. Stop if the chest does not rise and recheck the airway.

STOP AND CALL FOR HELP IF the chest does not rise after two attempts, there is no pulse, or the child is not improving.

Children most often stop breathing before the heart stops, so breaths alone can restore a failing circulation if they start early. The rate is slower than the adult rate because the lungs are smaller, and the rise of the chest tells you the breath was enough.

PROCEDURE: CPR FOR AN INFANT OR CHILD

Purpose: to keep blood moving in a child or infant who is unresponsive and not breathing normally. Scope: follows your training level and local protocol; the exact method your course teaches takes precedence.

1. Check that the scene is safe, tap the child's shoulder or the infant's foot and shout; if there is no response, shout for help.
2. If someone else is with you, send them to call emergency services and fetch an AED. If you are alone and have a phone, put it on speaker and call while you begin. If you are alone with no phone, give about 2 minutes of CPR first, then call and fetch an AED.
3. Place the child on a firm surface and check for breathing for no more than 10 seconds.

4. For a child, place the heel of one hand on the lower half of the breastbone. For an infant, either encircle the chest with both hands and press with both thumbs side by side, or use the heel of one hand, on the center of the chest just below the nipple line.
5. Press down at least one third of the depth of the chest (about 2 inches [5 centimeters] in a child and about 1.5 inches [4 centimeters] in an infant), at 100 to 120 a minute, and let the chest come fully back up after each push.
6. If you are trained, give 2 breaths after every 30 compressions; with 2 rescuers, give 2 breaths after every 15 compressions.
7. Use an AED as soon as it arrives, with child pads or a child setting if available (for a child under about 8 years). If none is available, use the AED with adult pads rather than no AED.
8. Continue until help takes over or the child responds.

> ⚠ **SAFETY**
>
> Do not press on the soft tissue under the chin or neck when you lift the jaw, and keep your hands on the breastbone, away from the upper abdomen. Keep an infant's head in a neutral position; tilt a child's head back only slightly past neutral.

STOP AND CALL FOR HELP IF a child or an infant is unresponsive and not breathing normally, or if you cannot clear an obstruction.

The steps differ from the adult sequence because cardiac arrest in children most often follows respiratory failure or shock and not a primary heart problem. Early breaths and oxygen matter more. An alone rescuer with no phone gives about 2 minutes of CPR before leaving to call. Compression depth is stated as a fraction of the chest so that it fits any size of child.

NURSING AND RESPONDER PRIORITIES. Match the technique to the age, and call early for any child who is not breathing normally.

See also: Entry 2.2 (choking); Entry 3.4 (CPR and AED); Entry 6.1 (child assessment).

6.3 Pregnancy Emergencies and Imminent Birth

In pregnancy the care you give the patient is also care for the baby. The principles of Parts 1 to 5 still apply, with a few adaptations.

CHANGES OF NORMAL PREGNANCY. In pregnancy the blood volume rises and the heart rate is higher at rest, so a pregnant patient can lose a good deal of blood before the usual signs of shock appear. The blood pressure often runs a little lower in the middle months. These changes mean that you cannot rely on the vital sign ranges of Entry 1.4. Treat the patient's own signs of distress as serious and tell the clinicians that she is pregnant and how far along she is.

POSITIONING. In the later months of pregnancy, the enlarged uterus can press on the large vessel that returns blood to the heart when the patient lies flat on her back, causing faintness and a fall in blood pressure.

Figure 25 shows left-lateral positioning; notice the rolled blanket behind her back that holds the position.



Figure 25. A pregnant patient lying on her left side.

PROCEDURE: POSITION A PREGNANT PATIENT ON THE LEFT SIDE

Purpose: to take the weight of the uterus off the large vessel that returns blood to the heart. Scope: follows your training level and local protocol.

1. Make the scene safe, explain what you will do, and ask permission.
2. Check airway and breathing, and call for help if she is unwell.
3. If you suspect a neck or back injury, do not turn her; call for help and follow your spinal motion restriction protocol (Entry 4.3).

4. Help her roll onto her left side, supporting her shoulder and hip.
5. Put a pillow under her head and a rolled blanket behind her back to hold the position.
6. Recheck her breathing, color and responsiveness often, and note the time she was placed on her side.
7. If she becomes unresponsive and is not breathing normally, lay her on her back on a firm surface and start CPR (Entry 3.4).

> ⚠ **SAFETY**
>
> Do not leave her flat on her back in late pregnancy. Do not tilt her for CPR, because compressions need a firm, flat surface.

STOP AND CALL FOR HELP IF she bleeds heavily, has severe pain, a seizure, a severe headache with vision changes, faints, or the baby is crowning.

The enlarged uterus can squeeze the large vein that carries blood back to the heart when she lies flat. Lying on her left side lifts that weight away, so more blood returns to the heart and the blood pressure improves.

EMERGENCIES. Call for help for any pregnant patient with vaginal bleeding, severe abdominal pain, a severe headache, changes in vision, swelling of the face and hands, a seizure, reduced movements of the baby, a fall or injury, or loss of fluid. A seizure in late pregnancy can be a sign of eclampsia, a dangerous complication of high blood pressure, and a severe headache with vision changes can be an early warning of it. Heavy bleeding is an emergency, and the signs of shock may appear late.

CPR IN PREGNANCY. Entry 3.4 described adult CPR. In advanced pregnancy, the same compressions apply, and the AED is used as usual. If a second rescuer is available, that person moves the uterus gently to the patient's left by hand while compressions continue. Call for help early, because the patient and the baby both depend on fast advanced care.

IMMINENT BIRTH. Birth is imminent when the patient feels an urge to push, the baby's head is visible at the vaginal opening (crowning), or contractions are very close together. Call emergency services and do not try to stop the birth or delay it. Make the patient comfortable, wash your hands or put on gloves, and have clean towels and a blanket ready. Let the baby emerge on its own, supporting the head gently as it comes. Do not pull on the baby or the cord. Once the baby is born, dry the baby with a towel, keep the baby warm against the patient's skin under a blanket, and check that the baby is breathing. Do not cut the cord unless you have been trained and protocol directs it, and do not pull on the cord to deliver the placenta. If the cord is visible before the baby, or the baby is not breathing after birth, tell the dispatcher at once and follow the instructions you are given.

RED FLAGS

Heavy bleeding, severe pain, a seizure, a severe headache with vision changes, a baby that is not breathing or is limp after birth, or a cord that comes out before the baby. Call for help at once.

NURSING AND RESPONDER PRIORITIES. Position on the left side, call early, and watch for shock.

See also: Entry 3.4 (CPR); Entry 3.5 (shock); Entry 6.2 (infant breathing).

6.4 Older Adults

Older adults are a large share of emergency patients, and the same illness often looks different in them. The core habits do not change. What changes is how much weight you give to the vague complaint, the quiet patient and the small change.

WHAT CHANGES. The body of an older adult has less reserve, so a patient may be seriously ill with few signs. A heart attack may bring breathlessness, weakness or tiredness with little or no chest pain. An infection may bring confusion and no fever. A fall may break a hip or bleed inside the skull with few signs at first. Older adults often take several medicines, and some of them blunt the body's response, for example a medicine that slows the heart may hide a fast pulse. Entry 3.2 gave the classic chest pain pattern. In older adults, and in women of any age, a heart attack may instead show as breathlessness, nausea, sweating or unusual tiredness, so treat unexplained breathlessness or sudden weakness as possible heart trouble and call early. A stroke may show as sudden confusion or a fall rather than a drooping face (Entry 5.2), and a serious abdominal problem may bring little pain, no fever and a soft belly (Entry 5.6), so the red flags of those entries apply even when the picture looks mild.

CONFUSION. New confusion is not a normal part of aging. When it comes on suddenly it is called delirium, and it is a sign of illness or another cause: infection, low blood glucose, low oxygen, medicines, pain, dehydration or a head injury. A patient with dementia may be confused all the time, so ask family or the caregiver what the usual state is and treat a change from that as an emergency.

FALLS. A fall is both an injury and a clue. Ask why the patient fell, whether they blacked out, whether they hit their head, and whether they take blood-thinning medicines. Entry 4.1 described the risk of bleeding after a minor fall.

MEDICINES. Ask for the list of medicines or look for it. Some medicines thin the blood, some lower the blood pressure, some blunt the heart rate, and some cause drowsiness or confusion on their own. A patient who has recently started a new medicine or changed a dose may be reacting to it. Do not stop or change any medicine yourself.

HEAT AND COLD. Older adults sense heat and cold less well and may not feel thirsty. Entry 4.6 described the signs; check the temperature of the home and ask how long the patient has been in it.

COMMUNICATION. Speak clearly, face the patient, allow extra time, check whether they can hear and see you, and involve them in decisions. Do not assume that a patient who is slow to answer cannot understand.

RED FLAGS

New confusion or drowsiness, a fall with a head strike on blood-thinning medicine, breathlessness with no clear cause, a fast or slow pulse with faintness, and any change from the usual state reported by family. Call for help at once.

NURSING AND RESPONDER PRIORITIES. Take vague symptoms seriously, ask what is usual for the patient, and call early.

See also: Entry 3.2 (chest pain); Entry 4.6 (heat and cold); Entry 5.1 (altered mental status); Entry 5.4 (sepsis).

6.5 Behavioral and Mental Health Emergencies

A person in a mental health crisis needs what every patient needs: a safe scene, a calm responder and a prompt call for help. The skills here are mostly skills of speaking and listening, and they protect both the patient and you.

WHAT IT IS. A behavioral emergency is a situation in which a person's thoughts, feelings or actions put them or others at risk. It may involve severe anxiety, panic, agitation, confusion, hearing or seeing things that are not there, thoughts of suicide, or aggression. Physical illness can cause the same behavior, such as low blood glucose, low oxygen, a head injury, infection or poisoning, so check the reversible causes of Entry 5.1 first. Do not assume that unusual behavior is only a mental health problem.

WHY PEOPLE BECOME AGITATED. Fear, pain, intoxication, withdrawal and an acute psychiatric illness can each cause agitation on top of the physical causes above, and a person may have more than one at once. A calm approach helps whatever the cause. Ask about medicines and substances, and about recent injury, as part of the history.

PANIC AND HEART TROUBLE. A panic attack can cause chest tightness, a racing heart and breathlessness, and several of these also occur in heart attacks and asthma. Do not assume that a patient with these symptoms is only anxious; check the vital signs and call for help if there is chest pain or the symptoms do not settle.

SAFETY FIRST. Assess the scene before you approach. Look for weapons, for exits, and for anyone else at risk. Keep a clear route out for yourself and the patient, stay at a safe distance, and do not stand between the patient and the door. If you feel threatened,

leave and call for help. Do not try to restrain a patient; this book does not teach restraint, and it carries serious risks for both of you.

DE-ESCALATION. Speak slowly and quietly, introduce yourself, and say what you want to do. Keep your hands visible and your posture relaxed. Listen more than you talk, and acknowledge the person's feelings without arguing about what they believe. Offer simple choices, such as where to sit. Avoid sudden movements, a crowd of onlookers and loud noise. Give the person time.

THOUGHTS OF SUICIDE. Ask directly and kindly whether the person is thinking of suicide. Asking does not put the idea into someone's head. If the answer is yes, stay with the person, take the concern seriously, and call for help. Use language that treats suicide as a health matter, such as "died by suicide", and avoid phrases that blame or glamorize. Do not leave the person alone, and remove means of harm from the area if you can do so safely. In the United States, people in crisis can call or text 988 to reach the national crisis line, and 911 is for immediate danger.

RED FLAGS

A person who talks about wanting to die or has a plan or the means, a weapon, threats to others, severe agitation with confusion, chest pain or breathing trouble with panic, or signs of poisoning. Call emergency services at once.

SAFETY

Do not argue with, shame or promise secrecy to a person at risk. Do not leave a person who may harm themselves alone. Leave the scene if it becomes unsafe and call for help.

STOP AND CALL FOR HELP IF the person has a weapon, is threatening to harm themselves or others, or you cannot keep the scene safe.

WHEN THE PERSON REFUSES HELP. A person in crisis may refuse care. Entry 1.5 set out consent. Stay calm, keep talking, do not argue and call for help, because decisions about whether someone may refuse, or be held for their own safety, rest with clinicians and the law of your state.

NURSING AND RESPONDER PRIORITIES. Check the scene, rule out medical causes, ask directly about suicide, and stay with the person.

See also: Entry 5.1 (altered mental status); Entry 5.5 (poisoning and overdose); Entry 6.6 (violence).

6.6 Abuse, Neglect and Interpersonal Violence

Responders see injuries and illnesses, and some of them are not accidents. The patient may be a child, an older adult or a partner who cannot ask for help, and you may be the only professional who sees the signs.

WHAT IT IS. Abuse is harm done to a person by someone with power over them, and it can be physical, sexual, emotional or financial. Neglect is the failure to provide care a person needs. Interpersonal violence includes violence between partners and other family members. These affect people of every age, sex and background.

SIGNS. Be alert when the injuries do not fit the explanation, when the story changes, when the delay in seeking care is long, when injuries are in different stages of healing, or when a patient

seems afraid of the person with them. In children, watch for bruising in an infant who cannot yet move around, bruises in unusual places, burns with clear outlines, a child who is withdrawn or very afraid, and signs of neglect such as poor hygiene or untreated illness. In older adults, watch for unexplained injuries, pressure sores, dehydration and poor nutrition, .

WHAT TO DO. Treat the injuries first. Speak to the patient privately if you can do so safely, and listen without judging. Record what you see and what the patient says in their own words, with the times, and do not interrogate. Do not confront the suspected abuser. Report your concern through the channels your service or school requires. In many states the legal duty to report belongs to you personally, so telling a supervisor may not be enough. In many places, health professionals are legally required to report suspected abuse of children and of vulnerable adults, and the rules differ by state and role. A report is a request for protection, and you do not need proof.

RED FLAGS

An injury that does not fit the story, injuries of different ages, a frightened patient or a controlling companion, delay in seeking care, and signs of neglect. Report your concern and call for help if anyone is in immediate danger.

SAFETY

Do not question the patient about abuse in front of the person you suspect, promise to keep a secret you cannot keep, or confront the suspected abuser. Your first aim is the patient's safety now.

RECORDING WHAT YOU SEE. Write the patient's words in their own words, the time, who was present, and each injury by place and size, for example a round bruise about 1 inch (2.5 cen-

timeters) across on the left upper arm. Describe; do not conclude. A note that says abuse is a conclusion, and a note that describes the bruise lets the next clinician judge.

NURSING AND RESPONDER PRIORITIES. Treat the injuries, listen privately, record facts, and report as your role requires.

See also: Entry 4.1 (injury patterns); Entry 6.1 (children); Entry 6.4 (older adults).

Illustrative Case

ILLUSTRATIVE CASE: NOT A REAL PATIENT

Sam was a nursing student on a placement in an emergency department triage area when a mother carried in her 18-month-old, Noor, who had been hot and fussy for two days and was not feeding. Noor lay against her mother's shoulder. Sam looked from the doorway before going near. Noor's eyes opened when her mother spoke, but she did not look around, and she let herself be handled without protest. Her breathing was fast, and the skin between her ribs pulled in with each breath. Her lips were pink, but her hands and feet looked pale and a little mottled.

Sam told the triage nurse what she had seen. The nurse took Noor's temperature and her breathing and heart rates, noted a slow return of color to Noor's fingertip, and moved her to the front of the line. Sam stayed with the mother, kept her calm, and asked when Noor had last drunk and wet a diaper. The mother said it had been many hours.

A doctor saw Noor within minutes. Afterward the triage nurse asked Sam what she had noticed first, and Sam answered in the order she had looked: how Noor seemed, how she breathed, and the color of her skin. She also gave the nurse the times the mother had mentioned, because Sam had written them down as she heard them.

Care Priorities

CARE PRIORITIES

Patients who do not fit the adult model share one trap: the usual numbers and the usual story mislead. Ask what is normal for this patient before you decide what is abnormal, and ask the person who knows, whether that is a parent, a caregiver or a relative. Look before you touch, because a frightened child, a person in crisis and a patient with dementia all react to how you approach, and that first reaction is part of your assessment. Give extra weight to the quiet patient, since a child who has stopped crying, a pregnant patient who feels well after a fall and an older adult with no complaint can each be the sickest person present. When the picture does not make sense, such as an injury that does not fit the story, write down what you saw and what you were told in the patient's own words, and pass it to the person who can act. Finally, check your scope before you act: equipment sizes and legal duties differ by age and by state, and when you are unsure the right move is to call early and stay.

Review Questions

1. A 2-year-old is held by her mother in the doorway. She is limp, does not look at you and has stopped crying. Which part of the pediatric assessment does this describe?

   A) Circulation to the skin.

   B) Work of breathing.

   C) Appearance.

   D) Airway, pulse and temperature.

2 How does choking relief in a conscious infant differ from that in an adult?

A) It uses chest thrusts in place of abdominal thrusts.
B) It uses abdominal thrusts with less force.
C) It uses blind finger sweeps.
D) It begins with CPR.

3 An alone rescuer finds an unresponsive child who is not breathing normally, with no phone to hand. According to Entry 6.2, which is correct?

A) Leave at once to call for help and return later.
B) Give 5 rescue breaths only and wait.
C) Wait until help arrives.
D) Give about 2 minutes of CPR, then call for help and fetch an AED.

4 Why is a pregnant patient in late pregnancy who feels faint helped onto her left side?

A) It lowers the risk of vomiting.
B) It moves the weight of the uterus off the large vessel that returns blood to the heart.
C) It slows labor.
D) It turns the baby head down.

5 A baby is born before help arrives. Which action is correct?

A) Pull on the cord to deliver the placenta.
B) Cut the cord at once with clean scissors.
C) Dry the baby, keep the baby warm against the patient's skin and check breathing.
D) Hold the baby upside down to clear fluid.

6 An 82-year-old who usually chats freely is drowsy and confused, and her daughter says this is new. What does this most suggest?

- **A)** An illness, such as infection, low glucose or low oxygen, that needs a cause to be sought.
- **B)** Normal aging.
- **C)** Dementia, so nothing more is needed.
- **D)** Tiredness from a poor night's sleep.

7 A person in crisis says they want to die. Which response matches Entry 6.5?

- **A)** Change the subject to avoid upsetting them.
- **B)** Promise to keep it secret.
- **C)** Tell them how much they have to live for.
- **D)** Ask directly and kindly about suicide, stay with them and call for help.

8 Short case. A 3-month-old is brought in by a parent. The infant has a bruise on the cheek and is unusually quiet and feeding poorly. The parent says the baby rolled off a sofa. List, in order, the first four things you would do.